

Policy on Quality Indicators, Data Analysis and Improvement

«Hospital Name»

Document control

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Prepared by	Quality Coordinator Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

«Hospital Name» identifies and monitors key indicators covering clinical structures, processes and outcomes, managerial structures, processes and outcomes, infection prevention and control activities, and patient safety activities.

The organisation verifies and analyses collected data, takes corrective and preventive actions where necessary, implements and evaluates improvements, and communicates feedback about care and service to staff.

1. Purpose

The purpose of this policy is to establish the process for identifying and monitoring quality indicators, verifying and analysing data, implementing improvements and communicating feedback about care and service.

This policy does not cover matters outside this quality-indicator and improvement process.

2. Scope

This policy applies to the identification, monitoring, verification, analysis and use of quality indicators at «Hospital Name».

It covers clinical, managerial, infection prevention and control, and patient safety indicators, together with data verification, improvement activities and communication of feedback about care and service.

3. Policy standards

3.1 Clinical indicators

The organisation identifies and monitors the priority aspects of patient care.

Any indicator mandated by the Government of India, State Government or NABH shall be monitored.

Some indicators could be monitored for patient assessment, safety and quality-control programmes of diagnostic services, medication management, use of blood and blood components, surgical services including use of anaesthesia, and mortality and morbidity indicators.

Every indicator shall have a defined numerator, denominator and multiplier.

For example, a Catheter-Associated Urinary Tract Infection (CAUTI) rate is calculated as (number of CAUTI events during the month divided by the number of urinary-catheter-days that month) multiplied by 1,000 — expressed as a rate per 1,000 catheter-days, following the current CDC/NHSN surveillance definitions.

Where appropriate, the definition of terms shall be provided.

3.2 Infection prevention and control indicators

The organisation shall identify and monitor appropriate key performance indicators suitable to it.

Any indicator mandated by the Government of India or NABH shall be monitored.

Some indicators that could be monitored include CAUTI, CLABSI, SSI and VAP.

These are illustrative examples and are not an exhaustive list.

Every indicator shall have a defined numerator, denominator and multiplier.

For healthcare-associated infections, the current surveillance definitions provided by the CDC's National Healthcare Safety Network shall be used.

3.3 Managerial indicators

The organisation identifies and monitors priority managerial activities.

Any indicator mandated by the Government of India, State Government or NABH shall be monitored.

Some indicators could be monitored for medication procurement, utilisation rates, patient and staff satisfaction, waiting time for consultation and diagnostics, and availability and content of medical records.

Every indicator shall have a defined numerator, denominator and multiplier.

3.4 Patient safety indicators

The organisation shall identify and monitor appropriate key performance indicators suitable to it.

Any indicator mandated by the Government of India, State Government or NABH shall be monitored.

Some indicators could be monitored for patient safety goals and risk management.

Every indicator shall have a defined numerator, denominator and multiplier.

3.5 Data verification

The data collected is verified from time to time, in response to queries, or when an unexplained trend occurs.

The data collected shall be authenticated by the quality team after going through every data point or by using random sampling.

The verification process shall also include whether the stated sampling strategy, where applicable, was adhered to.

Whenever errors are detected in the process of collecting data, they are corrected.

3.6 Data analysis and improvement opportunities

The data is analysed and, based on this, corrective and preventive actions are taken where necessary.

The organisation could consider developing benchmarks or acceptable quality levels based on national or international norms.

The organisation could undertake quality improvement projects when the benchmark is consistently not met or the trend is unfavourable.

If the organisation is consistently meeting its benchmark, it could reduce the frequency of monitoring.

3.7 Evaluation of improvements

The improvement activities carried out by the organisation shall have an evaluable outcome.

The outcome shall be documented.

3.8 Communication of feedback

Feedback about care and service is communicated to staff.

The feedback shall include rates, trends and opportunities for improvement.

It could also provide specific inputs to improve or reduce the rate. This could be in the form of a bulletin or newsletter.

Positive feedback about care and service is also communicated to staff.

4. Non-negotiable rules

1. Do not treat an indicator as complete without a defined numerator, denominator and multiplier.
2. Do not omit monitoring of an indicator mandated by the Government of India, State Government or NABH.
3. Do not authenticate collected data without the quality team going through every data point or using random sampling.
4. Do not omit verification of whether the stated sampling strategy was followed where a sampling strategy applies.
5. Do not leave detected errors in the data-collection process uncorrected.
6. Do not treat data analysis as complete where corrective and preventive actions are necessary but have not been taken.
7. Do not treat an improvement activity as complete without an evaluable outcome and its documentation.
8. Do not communicate feedback about care and service without including rates, trends and opportunities for improvement.

5. What we do

5.1 Identify and monitor clinical indicators

The organisation identifies and monitors priority aspects of patient care.

Government of India, State Government and NABH-mandated indicators are monitored.

The organisation could monitor indicators relating to patient assessment, diagnostic-service safety and quality-control programmes, medication management, blood and blood components, surgical services including anaesthesia, and mortality and morbidity.

Each indicator shall have a defined numerator, denominator and multiplier. Where appropriate, definitions of terms shall be provided.

5.2 Identify and monitor infection prevention and control indicators

The organisation shall identify and monitor appropriate infection prevention and control key performance indicators suitable to it.

Government of India and NABH-mandated indicators shall be monitored.

CAUTI, CLABSI, SSI and VAP are examples of indicators that could be monitored.

Each indicator shall have a defined numerator, denominator and multiplier.

For healthcare-associated infections, the current surveillance definitions provided by the CDC's National Healthcare Safety Network shall be used.

5.3 Identify and monitor managerial indicators

The organisation identifies and monitors priority managerial activities.

Government of India, State Government and NABH-mandated indicators shall be monitored.

The organisation could monitor indicators relating to medication procurement, utilisation rates, patient and staff satisfaction, waiting time for consultation and diagnostics, and availability and content of medical records.

Each indicator shall have a defined numerator, denominator and multiplier.

5.4 Identify and monitor patient safety indicators

The organisation shall identify and monitor appropriate patient safety key performance indicators suitable to it.

Government of India, State Government and NABH-mandated indicators shall be monitored.

The organisation could monitor patient safety goals and risk management.

Each indicator shall have a defined numerator, denominator and multiplier.

5.5 Verify collected data

The data collected is verified from time to time, in response to queries or when an unexplained trend occurs.

The quality team shall authenticate the collected data after going through every data point or by using random sampling.

Where applicable, the verification includes whether the stated sampling strategy was followed.

Detected errors in data collection are corrected.

5.6 Analyse data and identify improvement opportunities

The data is analysed and corrective and preventive actions are taken where necessary.

The organisation could consider developing benchmarks or acceptable quality levels based on national or international norms.

The organisation could undertake quality improvement projects when the benchmark is consistently not met or the trend is unfavourable.

If the organisation is consistently meeting its benchmark, it could reduce the frequency of monitoring.

5.7 Implement and evaluate improvements

Improvement activities carried out by the organisation shall have an evaluable outcome.

The outcome shall be documented.

5.8 Communicate feedback

Feedback about care and service is communicated to staff.

The feedback shall include rates, trends and opportunities for improvement.

Specific inputs to improve or reduce the rate could also be provided, including through a bulletin or newsletter.

Positive feedback about care and service is also communicated to staff.

6. Stop-work authority

6.1 Infection prevention and control indicator trigger

Absence of infection prevention and control indicator monitoring is a CORE non-compliance. Where infection prevention and control indicator monitoring is absent, the deficiency is a stop-work trigger under this policy and shall be escalated for correction before normal compliance status is treated as restored.

6.2 Patient safety indicator trigger

Absence of patient safety indicator monitoring is a CORE non-compliance. Where patient safety indicator monitoring is absent, the deficiency is a stop-work trigger under this policy and shall be escalated for correction before normal compliance status is treated as restored.

7. Governance and responsibility

Role	Responsibility
Leadership	Supports identification and monitoring of clinical, managerial, infection prevention and control, and patient safety indicators. Supports the use of analysed data for corrective and preventive actions where necessary.
Quality team	Authenticates collected data after going through every data point or by using random sampling. Verifies adherence to the stated sampling strategy where applicable. Supports correction of detected data-collection errors.

Relevant departments and staff	Provide the data required for monitoring and participate in improvement activities. Receive feedback about care and service.
Responsible persons for improvement activities	Ensure that improvement activities have evaluable outcomes and that the outcomes are documented.

8. Quality monitoring

Monitoring area	What is monitored
Clinical indicators	Priority aspects of patient care and mandated indicators
Infection prevention and control	Appropriate key performance indicators and mandated indicators
Managerial indicators	Priority managerial activities and mandated indicators
Patient safety	Appropriate key performance indicators and mandated indicators
Indicator definition	Numerator, denominator and multiplier
Data verification	Quality-team authentication and sampling-strategy verification where applicable
Data errors	Correction of detected errors
Data analysis	Analysis leading to corrective and preventive actions where necessary
Improvement activities	Evaluable outcome and documentation
Feedback	Rates, trends and opportunities for improvement communicated to staff

9. Training and staff acknowledgement

Staff involved in quality-indicator monitoring, data verification, analysis, improvement activities and communication of feedback are familiar with the processes described in this policy.

Staff acknowledgement:

I have read the Policy on Quality Indicators, Data Analysis and Improvement of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to staff involved in quality-indicator monitoring, data verification, analysis, improvement activities and communication of feedback.

Feedback about care and service is communicated to staff.

11. Abbreviations

Abbreviation	Meaning
CAUTI	Catheter Associated Urinary Tract Infection
CLABSI	Central Line Associated Blood Stream Infection
SSI	Surgical Site Infection
VAP	Ventilator Associated Pneumonia
CDC	Centers for Disease Control and Prevention
NABH	National Accreditation Board for Hospitals and Healthcare Providers

12. Traceability table

This table is an index. It is not how the policy is organised. Note: PSQ.3.b and PSQ.3.d are CORE with no asterisk — neither requires documentation per the source inventory. The source draft incorrectly marked them CORE*; corrected here.

Objective Element	Level	Traceability to this policy
PSQ.3.a	Commitment	Sections 3.1 and 5.1 address identification and monitoring of priority clinical indicators, mandated indicators, and defined numerator, denominator and multiplier.
PSQ.3.b	CORE	Sections 3.2, 5.2 and 6.1 address identification and monitoring of infection prevention and control indicators, defined indicator measures, required surveillance definitions for healthcare-associated infections, and the CORE stop-work trigger.
PSQ.3.c	Commitment	Sections 3.3 and 5.3 address identification and monitoring of priority managerial indicators, mandated indicators, and

		defined numerator, denominator and multiplier.
PSQ.3.d	CORE	Sections 3.4, 5.4 and 6.2 address identification and monitoring of patient safety indicators, defined indicator measures, and the CORE stop-work trigger.
PSQ.3.e	Commitment	Sections 3.5 and 5.5 address data verification, quality-team authentication, sampling-strategy verification where applicable, and correction of detected errors.
PSQ.3.f	Commitment	Sections 3.6 and 5.6 address data analysis and corrective and preventive actions where necessary, while preserving the optional nature of benchmarks, quality improvement projects and reduced monitoring frequency.
PSQ.3.g	Commitment	Sections 3.7 and 5.7 address evaluable outcomes and documentation of improvement activities.
PSQ.3.h	Achievement	Sections 3.8 and 5.8 address communication of feedback including rates, trends and opportunities for improvement, together with positive feedback and optional specific improvement inputs.

13. Required Records/Evidence Checklist

Clinical indicators

- Current list of priority clinical indicators and their monitoring results.
- Government of India, State Government or NABH-mandated indicators being monitored.
- For each indicator, a defined numerator, denominator and multiplier.
- Definitions of terms used for indicators where appropriate.

Infection prevention and control

- List of appropriate infection prevention and control key performance indicators and their monitoring results.
- Government of India or NABH-mandated infection prevention and control indicators being monitored.
- For each infection prevention and control indicator, a defined numerator, denominator and multiplier.
- Current CDC National Healthcare Safety Network surveillance definitions used for healthcare-associated infections.
- CAUTI, CLABSI, SSI or VAP monitoring where the organisation has selected these as indicators.

Managerial indicators

- List of priority managerial indicators and their monitoring results.
- Government of India, State Government or NABH-mandated managerial indicators being monitored.
- For each managerial indicator, a defined numerator, denominator and multiplier.
- Monitoring information for selected areas such as medication procurement, utilisation rates, patient and staff satisfaction, waiting time for consultation and diagnostics, and availability and content of medical records.

Patient safety indicators

- List of appropriate patient safety key performance indicators and their monitoring results.
- Government of India, State Government or NABH-mandated patient safety indicators being monitored.
- For each patient safety indicator, a defined numerator, denominator and multiplier.
- Patient safety goals or risk-management indicators where the organisation has selected them.

Data verification

- Quality-team authentication of collected data through review of every data point or random sampling.
- Checks showing that the stated sampling strategy was followed where applicable.
- Corrected data where errors were detected during data collection.

Data analysis and improvement

- Data-analysis reports showing analysis of the collected indicators.
- Corrective and preventive actions taken where necessary following data analysis.
- Benchmarks or acceptable quality levels where the organisation has chosen to develop them.
- Quality improvement projects where the organisation has chosen to undertake them because a benchmark was consistently not met or the trend was unfavourable.

- Documented evaluable outcomes for improvement activities.

Feedback and communication

- Feedback information communicated to staff showing rates, trends and opportunities for improvement.
- Specific inputs for improving or reducing rates where these have been developed.
- Bulletins or newsletters where this communication method is used.
- Positive feedback about care and service communicated to staff.

14. References

- National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. PSQ.3.
- Centers for Disease Control and Prevention. (n.d.).
<https://www.cdc.gov/nhsn/psc/index.html> National Healthcare Safety Network (NHSN) — Patient Safety Component surveillance definitions for healthcare-associated infections.

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked [Hospital to define] must be replaced with the hospital's own decision; a document issued with those markers left in place is not an approved policy.

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